

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Doxycycline or Azithromycin for the treatment of Chlamydia

Summary of NICE / NICE CKS Guidelines for Chlamydia

Overview

- **Chlamydia trachomatis** is the most common **bacterial sexually transmitted infection (STI)** in the UK.
- Often **asymptomatic** (up to 70% of women and 50% of men).
- Untreated infection can lead to complications: **pelvic inflammatory disease (PID)**, **infertility**, **ectopic pregnancy**, and **epididymo-orchitis**.

Assessment

Symptoms (when present):

- **Women:** vaginal discharge, intermenstrual/postcoital bleeding, dysuria, pelvic pain.
- **Men:** urethral discharge, dysuria, testicular pain (epididymo-orchitis).
- **Both:** rectal pain/discharge (if rectal infection), conjunctivitis (rare).

Risk factors:

- Age under 25.
- Multiple sexual partners.
- New sexual partner in the last 12 months.
- Inconsistent condom use.

Testing

- Use **NAAT (nucleic acid amplification test)** – highly sensitive.
 - **Women:** self-taken vulvovaginal swab (preferred), or endocervical swab.
 - **Men:** first-catch urine sample.
- Offer **rectal/pharyngeal swabs** based on sexual practices.

Management

First-Line Treatment

- **Doxycycline 100 mg twice daily for 7 days**
 - Effective for urogenital, rectal, and oropharyngeal chlamydia.
 - Avoid in pregnancy.

Alternatives (e.g. pregnancy):

- **Azithromycin 1 g stat, then 500 mg once daily for 2 days** (*some guidelines suggest single 1 g dose – follow local policy*).
 - **Erythromycin** or **amoxicillin** may be used if doxycycline and azithromycin unsuitable.
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Partner Notification and Treatment

- Notify and treat **all sexual partners** from the **preceding 6 months**.
 - Partners should be tested and **empirically treated** without waiting for results.
 - Advise **no sex until both patient and partner(s) have completed treatment**.
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Follow-Up

- **Test of cure:**
 - Not routinely needed unless:
 - Pregnancy
 - Non-standard treatment
 - Persistent symptoms
 - Rectal infection
 - If required, do at least **3 weeks after treatment** to avoid false positives.
 - **Retesting recommended after 3 months** to detect reinfection, especially in under-25s.
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Prevention and Education

- Promote **condom use**.
- Offer **full STI screening**, including HIV and syphilis.
- Offer **contraceptive advice** as needed.
- Under-25s should be screened **annually** or with each new partner (as per National Chlamydia Screening Programme).

Patient Group Direction

for the supply/administration of

Doxycycline 100mg

for the treatment of

Chlamydia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Doxycycline is indicated for the treatment of uncomplicated genital chlamydia trachomatis infection in adults and adolescents aged 15 years and over.
Inclusion criteria	- Individuals aged 15 years and over. - Confirmed or strongly suspected diagnosis of genital chlamydia infection. - Informed consent obtained. - No contraindications to doxycycline.
Exclusion criteria	- Known hypersensitivity to tetracyclines. - Pregnant or breastfeeding individuals. - Severe hepatic insufficiency. - Known or suspected complicated infection (e.g., PID). - Inability to comply with 7-day regimen or swallow tablets.
Cautions	- Advise on taking with water and remaining upright for 30 minutes to avoid oesophageal irritation. - Avoid sun exposure during and after treatment due to risk of photosensitivity. - Counsel on abstaining from sexual activity until treatment and partner treatment are completed. - Use effective contraception during and for 7 days after completing the course.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Doxycycline 100mg capsules
Legal category	POM
Storage	Store below 25°C. Protect from moisture and light.
Route/method of administration	Oral administration with a full glass of water while sitting or standing.
Dose and frequency	100 mg twice daily for 7 days.
Quantity to be administered and/or supplied	14 capsules or tablets.
Maximum or minimum treatment period	7 days continuous course.
Adverse effects	Nausea, vomiting, diarrhoea, abdominal pain, photosensitivity, oesophageal irritation. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
25/8/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Azithromycin 500mg

for the treatment of

Chlamydia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Azithromycin is indicated for the treatment of uncomplicated genital chlamydia trachomatis infection in adults and adolescents aged 15 years and over where doxycycline is unsuitable or contraindicated.
Inclusion criteria	- Individuals aged 15 years and over. - Confirmed or strongly suspected diagnosis of genital chlamydia infection. - Informed consent obtained. - Doxycycline unsuitable or contraindicated.
Exclusion criteria	- Known hypersensitivity to macrolides. - History of QT prolongation or taking interacting QT-prolonging drugs. - Severe hepatic impairment. - Pregnant or breastfeeding unless assessed as suitable by a prescriber. - Concurrent use of ergot derivatives.
Cautions	- Use with caution in patients with mild to moderate liver or kidney impairment. - Advise patient not to take antacids 2 hours before or after dose. - Counsel on abstaining from sexual activity for 7 days after treatment and until partners are treated. - Reinforce need for test of cure if symptoms persist or in pregnancy.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Azithromycin 500mg
Legal category	POM
Storage	Store below 25°C. Protect from moisture and light.
Route/method of administration	Oral administration with or without food.
Dose and frequency	1 g as a single dose (two 500 mg tablets taken together).
Quantity to be administered and/or supplied	2 tablets (total 1 g).
Maximum or minimum treatment period	Single dose treatment.
Adverse effects	Nausea, vomiting, diarrhoea, abdominal pain, flatulence, headache. Rare: QT prolongation or allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
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